

CSU disease activity band shift after long-term treatment with remibrutinib in the phase 3 REMIX-1 and REMIX-2 studies

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KEY FINDINGS AND CONCLUSIONS

- Remibrutinib **reduced CSU disease activity** as early as **week 1** in patients with CSU, and the **fast response** was **sustained** over **long-term** treatment for **52 weeks**
- Of note, treatment transition from placebo to remibrutinib resulted in **similar fast improvements, with well-controlled and complete response levels being comparable to remibrutinib patients** at **week 52**
- **Remibrutinib** has the potential to become a novel oral treatment option that provides **fast (as early as week 1)** and **sustained improvements** in disease activity in patients with **CSU**

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INTRODUCTION

- Remibrutinib, a novel, highly selective, oral, Bruton's tyrosine kinase inhibitor, has previously shown superior efficacy versus placebo at week 12 and a favourable safety profile in the 24-week double-blind period of the pivotal phase 3 REMIX-1 and REMIX-2 studies in patients with chronic spontaneous urticaria (CSU) who remained symptomatic despite treatment with second-generation H1-antihistamines¹
- In a previously presented REMIX analysis up to week 24, a reduction in CSU disease activity was observed as early as week 1 with remibrutinib, sustained up to 24 weeks of treatment in the target population of patients with moderate to severe CSU disease activity at baseline²

OBJECTIVE

- The objective of this analysis was to explore the shift in weekly Urticaria Activity Score (UAS7) bands after treatment with remibrutinib versus placebo up to **week 52 on a patient level** in the REMIX studies

METHODS

Study design

- REMIX-1 and REMIX-2 are two identical, global, double-blind, placebo-controlled phase 3 studies of remibrutinib 25 mg twice daily (bid) administered orally. Adult patients with CSU who remained symptomatic despite treatment with second-generation antihistamines were randomised 2:1 to oral remibrutinib 25 mg bid or placebo for 24 weeks, followed by an open-label treatment with remibrutinib 25 mg bid for 28 weeks (patients on placebo transitioned to remibrutinib at week 24)¹

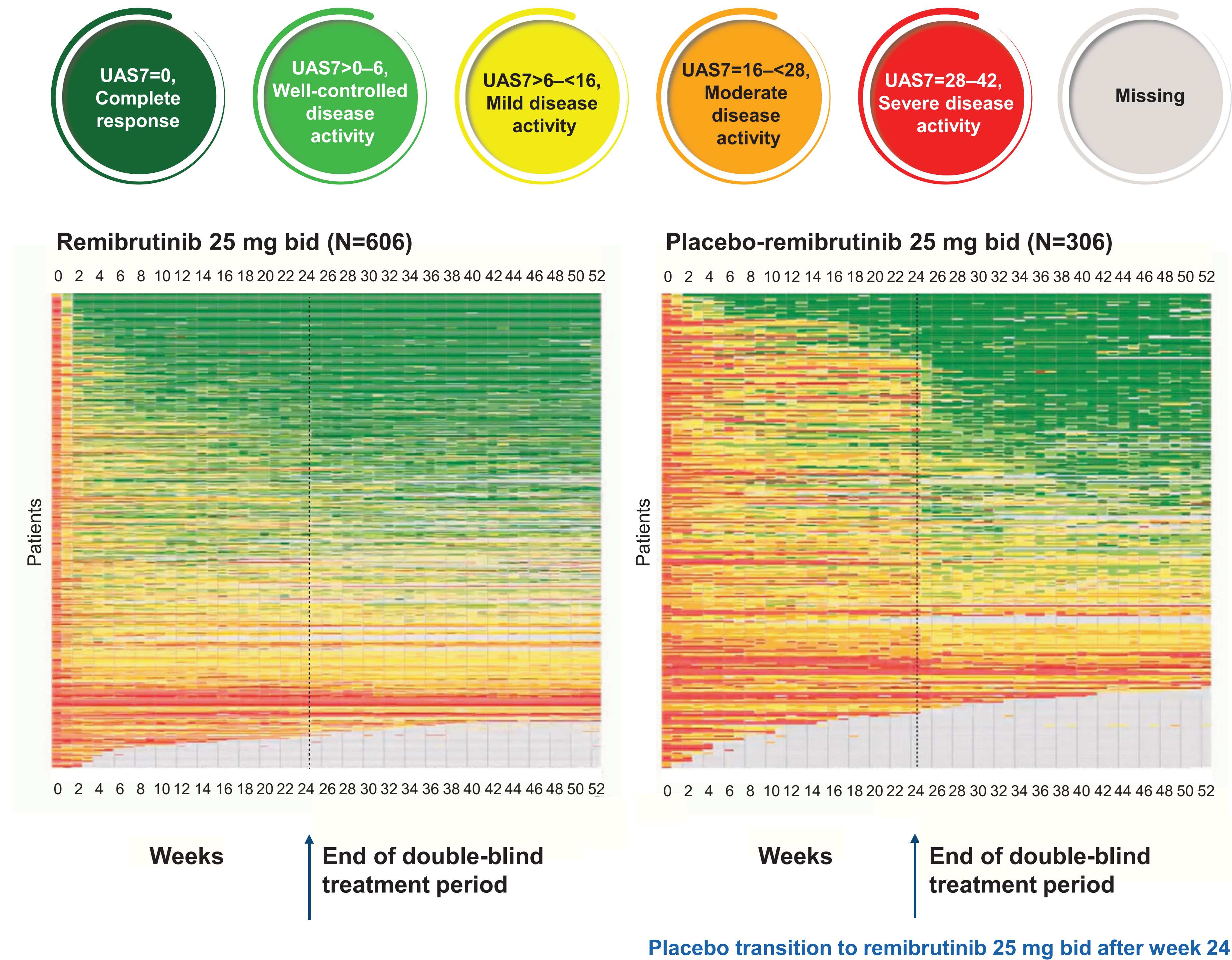
Study assessments and data analysis

- CSU disease activity was categorised into five bands, based on UAS7 (**Figure**)
- This post hoc analysis assessed the proportion of patients who experienced a shift in CSU disease activity from baseline to week 52 after treatment
- In addition, patients' individual UAS7 band shifts per week, up to week 52, were visualised in swimmer plots. Each patient is represented by a horizontal line, with each UAS7 band achievement represented by a colour, as indicated in the **Figure**

RESULTS

- This pooled analysis included randomised patients who received at least one dose of **remibrutinib 25 mg bid (N=606)** or **placebo for 24 weeks (N=306)** in the **REMIX-1 and REMIX-2 studies**
- Disease severity at baseline was similar among patients in the remibrutinib and placebo treatment arms; **215 (35.5%)** and **386 (63.7%)** patients from the **remibrutinib** arm and **122 (39.9%)** and **181 (59.2%)** from the **placebo** arm had **moderate** and **severe** CSU disease activity, respectively
- Overall, patients treated with **remibrutinib** vs **placebo** experienced substantial improvements in CSU disease activity and moved to a **lower disease activity band** as early as **week 1**, with more patients remaining in lower disease activity bands up to **week 24 (Figure)**
 - Patients on placebo **transitioned to remibrutinib 25 mg bid** after **week 24** and moved to a **lower disease activity band** as early as **week 1** after the transition and remained in the lower disease activity bands up to week 52, in line with patients who were on remibrutinib throughout (**Figure**)
- In the remibrutinib treatment arm, while **63.7% of patients** were in the **severe band** at baseline, the number dropped to **24.9%, 17.2%, 9.1%, 7.8%** and **8.1%** at **weeks 1, 2, 12, 24** and **52**, respectively
- Similarly, of the **35.5%** of patients in the **moderate band** at baseline, the number dropped to **30.7%, 24.1%, 10.6%, 7.9%** and **7.3%** at **weeks 1, 2, 12, 24** and **52**, respectively
- There were no patients in the **well-controlled** and **complete response** disease bands at baseline; however, the numbers for the **well-controlled** (UAS7≤6) and **complete response** (UAS7=0) groups combined increased with remibrutinib vs placebo to **11.7% (71/606)** vs **0.7% (2/306)** and **31.5% (191/606)** vs **4.2% (13/606)** at **weeks 1** and **2**, consistently improving up to **week 24 (48.5% [294/606] vs 28.4% [87/306])**
 - Notably, the proportion of patients receiving remibrutinib who **showed complete response increased** from **0.3%** at **week 1** to **16.2%** at **week 2**, with continued improvements up to **week 52 (35.1%)**

Figure. Swimmer plot of the disease activity band shift based on UAS7 scores from baseline to week 52 (pooled full analysis set; observed data)



Each patient is represented by a horizontal line. bid, twice daily; N, number of patients; UAS7, weekly Urticaria Activity Score.

- By the **end of week 52**, patients who had **transitioned to remibrutinib** from placebo, after **week 24**, had achieved similar band shifts as patients who had been on remibrutinib for 52 weeks

References

1. Metz M, et al. Oral presentation at: EACCI 2024; 31 May–03 June 2024; Valencia, Spain. Abstract 100107.
2. Maurer M, et al. Oral presentation at: EACCI 2024; 31 May–03 June 2024; Valencia, Spain. Abstract 000439.

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